



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
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June 9, 2026

VIA ELECTRONIC MAIL

Evan Escobar
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Proposed Contract Termination with Valley Children's Hospital
Block Transfer Filing No. 2026-HOSP-293
Filed on March 17, 2026

Dear Mr. Evan Escobar:

The Department of Managed Health Care (Department) is reviewing the information in the above-referenced Block Transfer Filing (Filing) submitted by **Blue Cross of California** (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove **Valley Children's Hospital** (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently **July 15, 2026**.

Please review the following comments and feel free to contact me if you would like to discuss any of the issues before filing the Plan's response.

1. The Plan indicates in its response to Comment Letter dated June 5, 2026, that the Plan received a two-week extension with a new termination date of July 15, 2026, to provide time to finalize the final documents for execution for the new agreement. Please confirm whether the Plan intends to withdraw this filing.
2. For Provider Group, LaSalle Medical Associates the Plan confirms there are 91,823 assigned enrollees. Please confirm, summarize, or provide the following:
 - a. Whether LaSalle Medical Associates has routine access to the Terminating Hospital,

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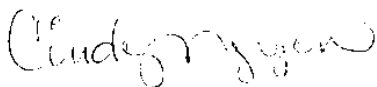
- b. The pending contract termination impacts 50,000 or more enrollees, please submit the relevant portions of the Provider Contract(s) that relate to continuity of care and transition of care.
3. For each affected provider group that utilizes the Terminating Hospital, please summarize or confirm the following:
 - a. Whether the provider group has routine access to the Terminating Hospital,
 - b. The ability of the provider group to redirect enrollees to the Alternate Hospitals:
 - i. Whether there is any specialty for which all of the associated specialists have admitting privileges **only** at the Terminating Hospital and not at any of the Alternate Hospitals,
 1. If yes, please identify the specialty and explain how Plan enrollees assigned to the provider group will receive care and services for that specialty following contract termination.
 - ii. If the provider group does not have any primary care physicians or specialists with admitting privileges to one or more Alternate Hospitals, please explain how its assigned enrollees will be admitted to the Alternate Hospital(s) (e.g. via hospitalists, Plan admission process, etc.).
 1. If hospitalists are utilized, please indicate:
 - a. The name of the group,
 - b. The number of associated hospitalists, and
 - c. If the hospitalists can admit enrollees for any medically necessary services.
4. Please confirm whether there are enrollees who have a residence or workplace within 15 miles or 30 minutes of a contracted hospital prior to termination, but would have a residence or workplace beyond 15 miles or 30 minutes of a contracted hospital post-termination.
5. Please confirm whether the Terminating Hospital offers Tertiary or Quaternary Care Services, Cancer Services, and Burn Services. If yes, please confirm which Alternate Hospital(s) provides each service(s).
6. The Plan indicates the Terminating Hospital is a transplant center. Please provide the following:
 - a. Number of affected enrollees who have or are currently receiving transplant-related services,
 - b. A list of transplant patients (no patient names, just number of patients) identifying the type of transplant, and the stage of the transplant (e.g. undergoing evaluation, on a transplant waiting list, scheduled for procedure or receiving post-transplant care),

- c. Whether there is a separate contract for transplant services,
 - i. If there is a separate contract for transplant services, please confirm whether the separate transplant contract is terminating at the same time or within the next six months,
 - d. Confirmation that current and future transplant enrollees will be able to continue receiving services through the Plan at the Terminating Hospital following contract termination pursuant to agreed upon contractual terms, or
 - e. If no contract in 6.d. exists, confirmation that the Plan has considered allowing affected enrollees in the transplant process to continue to receive those services at the Terminating Hospital, and the reasons for the Plan's decision, if one has been reached.
7. In the Plan's response to Comment Letter dated March 25, 2026, the Plan indicates the following high-level pediatric services are available at the Terminating Hospital: Level IV NICU/Regional NICU services, PICU services, Level II Pediatric Trauma services, and pediatric cancer services. Of those high-level services, please state which of those services are available to children at the Alternate Hospitals regardless of scope or designation.
8. Please identify the number of enrollees who have a procedure scheduled and authorized by the Plan at the Terminating Hospital post-termination.

Please submit the Plan's response by **June 11, 2026**, via the Block Transfer portal as a "Response to Additional Comment Letter."

Please contact me if there are any questions regarding the above.

Sincerely,



Cindy Nguyen
Analyst II
Office of Plan Monitoring